

Dr Bola Akinola: The Clinical Community Portal

A doctor-only portal that turns referrals into a community, and a community into a talent pipeline

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Why this is more than a feature

The referrer-engine playbook argued that elective specialist surgery is won on trusted relationships with other doctors, not advertising. The Clinical Community Portal is that engine made digital and durable. It takes the one asset a better-funded competitor cannot copy, a community of doctors who trust Dr Bola, and gives it a home, a memory, and a ladder.

It does four jobs at once, and each reinforces the others:

- **Refer.** A dead-simple way for doctors to send patients and get them back, with the loop closed automatically. This is the referrer engine, operationalised.
- **Discuss.** Multidisciplinary team meetings (MDTs) where doctors bring their hard cases and Dr Bola leads the room. This is the complex-case authority made social, and it is a referral funnel, a teaching moment and a trust-builder in one ritual.
- **Learn.** Teaching, technique, and case libraries from Dr Bola. CPD that keeps him front of mind between referrals.
- **Progress.** A path from referring doctor to fellow, feeding the OrthoSurplus academy and its roaming arthroplasty unit with trained surgeons.

Put together, it is a moat. A rival can open a smarter clinic; they cannot quickly build a living community of referring doctors and fellows that took years of generosity to earn.

1. The doctor's ladder

The portal is organised as a progression. A doctor climbs it at their own pace, and each rung deepens the relationship and moves them closer to the practice and, ultimately, the group.

Rung	Who	What they can do	What it gives the practice
1. Member	Any verified doctor	Register (license-verified), access resources, see the directory	A known, growing pool of referrers
2. Referrer	Members who refer	Submit referrals, track status, receive return letters, see their patients' outcomes	The referral pipeline, with the loop closed by design
3. MDT participant	Engaged referrers	Join the regular complex-case MDT, submit cases for discussion	Reputation, referrals, and a reason to stay engaged
4. Learner	Any member	Watch teaching and technique, attend live sessions, earn CPD	Front-of-mind authority between referrals
5. Fellow	Selected, committed doctors	Enter the structured arthroplasty fellowship toward becoming a roaming-unit associate	The talent pipeline for OrthoSurplus



The genius of the ladder is that it serves two strategies at once. Rungs 1 to 4 feed **Dr Bola's referral practice**. Rung 5 feeds the **OrthoSurplus academy and roaming unit**. The same community produces both patients and surgeons.

2. The MDT is the heart of it

Of everything here, the multidisciplinary team meeting is the most valuable and the most distinctive, so it deserves its own note.

A regular (say monthly) virtual complex-case round, where any member doctor can submit a difficult case (de-identified) and Dr Bola, with the room, works through it. It is simple to run and quietly powerful:

- **It is the complex-case authority in action.** Doctors see, first-hand, that this is the surgeon for the hard cases. No advertisement earns that.
- **It is a referral funnel.** Many cases discussed become cases referred, naturally and without a sales pitch.
- **It is teaching.** Every attendee learns, which builds loyalty and CPD value.
- **It is community.** A recurring room where the region's orthopaedic and related doctors gather around Dr Bola becomes, over time, his network and his moat.

Start it small and human (a scheduled video call plus a case-submission form) and formalise as it grows. It can launch long before the full portal is built.

3. Feature set, by rung

Member (the front door)

- Doctor registration with verification (MDCN license check), so the community stays genuinely professional.
- Profiles and a member directory.
- A resource library (protocols, referral guidance, patient-education leaflets to share).

Referrer (the engine)

- A fast referral form (under a minute, mobile-first).
- Referral status tracking, so the referrer always knows where their patient is.
- Automated return-letters after assessment and after treatment: the referrer-engine's non-negotiable, "return every patient," done by the system every time.
- Visibility of their referred patients' outcomes (with consent), which is what earns the next referral.

MDT participant

- MDT calendar and registration.
- De-identified case submission (NDPR-compliant by design).
- Live session hosting or integration, plus a searchable archive of past discussions.

Learner

- Video library: technique, teaching, recorded MDTs and talks.
- Live session calendar with Dr Bola.
- CPD tracking and certificates.

Fellow (the apex, OrthoSurplus academy)

- Fellowship application and selection.



- Structured curriculum, a surgical logbook, and progression tracking toward roaming-unit associate.
- Co-branded with OrthoSurplus as the training-to-practice pathway.

Behind the scenes (admin)

- A referral dashboard: who referred whom, loop-closure status, outcomes, thank-yous, case value. This is the referrer-engine tracking system, live.
- Engagement analytics to spot the doctors worth cultivating and the fellowship candidates worth recruiting.
- WhatsApp and email notifications throughout, because that is how Nigerian doctors actually communicate.

4. Whose portal is it? The brand boundary

We established three distinct brands (Dr Bola, Osteon, OrthoSurplus). The portal respects them:

- **The portal is Dr Bola's**, on bolarinwaakinola.com. The community, the MDTs and the teaching are built around his clinical leadership. This is his primary-brand asset and his moat.
- **Referrals flow to Dr Bola** (and, as the roaming unit matures, to the group's other anchors and trained associates).
- **The fellowship (rung 5) is OrthoSurplus's academy**, co-branded at that rung as the formal training-to-practice pathway. A doctor climbs Dr Bola's community ladder and, at the top, steps into OrthoSurplus's training engine.

So the portal is the on-ramp; OrthoSurplus is where the most committed doctors are trained into the group. Clean brands, one continuous journey.

5. Build it in phases (do not overbuild)

The temptation is to build all five rungs at once. Do not. The referral layer alone delivers most of the value, and the MDT needs almost no technology to start. Sequence:

Phase 1, the referral MVP (highest value, lowest build). Doctor registration and verification, the referral form with status tracking, automated return-letters, and a resource library. This operationalises the referrer engine and is genuinely useful from day one.

Phase 2, the MDT. Launch the monthly complex-case round (a scheduled call plus a case-submission form to begin), then add the archive and CPD. Much of this can run before it is fully productised.

Phase 3, the learning hub. The video library, live-session calendar and CPD tracking, once there is content worth housing (recorded MDTs are the first content).

Phase 4, the fellowship module. Application, curriculum and logbook, built alongside the OrthoSurplus academy as the roaming unit takes shape.

Each phase stands on its own and earns the next.

6. Build-versus-leverage, and compliance

- **Reuse before rebuild.** C4A already operates platform infrastructure and CadreHealth, a clinician network with verification, profiles and learning already solved. Before commissioning a bespoke portal, we should scope what the referral, directory, learning and fellowship layers can borrow from existing C4A systems rather than build from scratch. This could cut the build materially. To decide.



- **Verification.** MDCN license verification at registration keeps the community professional and credible, and protects the brand.
- **Data protection.** MDT case submissions and referral data carry patient information. De-identification for MDTs and NDPR-compliant handling of referral data are requirements, not options, and must be designed in from Phase 1.
- **Dr Bola's time is the real cost.** MDTs and teaching take his hours. That is not overhead; it is the reputation-building work itself, and the portal is what turns each hour into durable community and referrals rather than a one-off favour. But it must be paced so it is sustainable.

7. How it ties the whole strategy together

This is the piece that makes the separate documents one system:

- The **referrer-engine playbook** becomes a live platform, not a manual habit.
- The **complex-case positioning** gets its recurring public proof, the MDT.
- The **Blue Ocean transparency move** has a home: outcomes shown to referrers, cases discussed openly, learning shared.
- The **OrthoSurplus academy and roaming unit** get their recruitment and training pipeline.
- And Dr Bola's **personal brand** gains the one thing that compounds fastest and copies slowest: a community of doctors who trust him.

One portal, and every part of the strategy has somewhere to live and grow.

8. Open decisions

1. **Build vs leverage:** how much the portal borrows from existing C4A and CadreHealth infrastructure versus a bespoke build. This shapes cost and timeline more than anything else.
2. **MDT cadence and format:** monthly to start, virtual, and who beyond Dr Bola sits on the panel.
3. **Fellowship design and credential:** the curriculum, selection, and what a "fellow" formally is, built with the OrthoSurplus academy.
4. **Co-branding at the fellowship rung:** exactly how Dr Bola's community and OrthoSurplus's academy are presented together.

9. What we need to move

- A decision on build-versus-leverage (open decision 1), which unblocks a real build plan and cost.
- Dr Bola's commitment to a first MDT date; it can launch almost immediately and starts compounding at once.
- The referral form fields and the return-letter templates (these come straight from the referrer-engine playbook).

The recommended first step is deliberately small and high-value: launch the MDT and stand up the Phase 1 referral MVP. Both begin building the community and the pipeline long before the full portal exists.